

I: The first question is about familiarity with AI. How familiar are you with AI technologies in general?

P: Not very much. I know ChatGPT, but that's about it. I'm not very tech-savvy.

I: Have you used ChatGPT?

P: Yes.

I: For what kinds of things?

P: For study work and for my work right now, mostly for advice and for trip planning.

I: What type of advice?

P: If I observe a behavior in a child that I don't have much knowledge about, I consult ChatGPT and ask for ideas about what interventions might be useful.

I: So you are not using AI within your organization?

P: No, they are very old school.

I: Did you disclose to your clients that you sometimes use AI for consultation?

P: No, I don't think so.

I: On a scale from 1 to 10, how familiar do you consider yourself with AI?

P: Probably a 3 or a 4.

I: This question is about diagnostics and assessment use. What are your thoughts on AI being used in diagnostic or assessment processes?

P: I think it can be quite useful because it's more objective than humans are, but it should be used in parallel with a human professional, not on its own. I think there is a lot of potential there.

I: Do you currently do diagnostics yourself?

P: No. I'm a psychologist, not a psychotherapist, so I'm not allowed to do diagnostics and testing.

I: You still think it might be useful, and you're saying it's more objective?

P: Yes, because sometimes people put labels very easily. I have a colleague who, as soon as she sees a child, says something like, that child is surely going to develop a personality disorder, and I don't see a lot of grounds or evidence for that yet. I think if that were an AI, it

wouldn't draw those conclusions so easily. It would use more objective parameters and not just a gut feeling.

I: So you feel that sometimes psychologists just think they already know what's up and come up with a story of boxes and labels.

P: Yeah, definitely. I think AI would be more cautious with that, and use more scientific parameters rather than just gut feeling.

I: Do you know if you're doing any preliminary screening before the intakes?

P: No, I don't do that. I have a psychologist who is my subordinate and she does that. I just get the children.

I: And how does the screening go?

P: What they do is that a school or a youth worker can hand in an application for a child. Then the psychologist at the institution goes through the application and invites the family, the child, and the youth worker or whoever submitted the application for an interview. They ask background questions and discuss goals for the time with us, goals of the child, goals of the person submitting the application, and the parents.

I: That's done in person, right?

P: Yes, that's done in person, not with AI at all.

I: It's also not like a questionnaire they fill out, it's more like a conversation.

P: I think there is sort of a questionnaire, but it's more about what types of problems the child has, what their strengths and resources are. I'm not sure when they deliver that to the family or the youth worker or the school, but we get a folder with this information when the children arrive.

I: Have you considered AI for doing those screenings?

P: I haven't, because I don't do the screening and I'm not entirely in touch with the people who do that part of the job. I think they want to make some changes in the organization to make things more digitalized, but I'm not sure if that also applies to the screening.

I: What about summarizing patient information? In an intake you talk a lot, and afterwards you probably have to summarize quite a lot. Do you think AI would be able to do that?

P: I think that would save a lot of time, because documenting is a lot of work and takes a lot of effort. I don't do it with AI, I just summarize my notes, but I think it would be much more convenient. I just wouldn't know how that could be implemented because of data protection. My organization is very strict about that, so you need to be really careful how you store information about the children.

I: So you're a little bit reluctant because of privacy issues.

P: Yeah, they are.

I: The next question is about monitoring and progress tracking. How do you view the role of AI in tracking patient progress over time?

P: I think it could also be very useful, because you have more objective tracking and information collection that can be stored more efficiently and compared. But what's complicated is the parameters you choose. What is progress? What is success? How do you measure that? Right now I don't really see a way how that could be done, but I'm also not so much into research anymore, so I don't know how far things are in that regard.

I: Do you do any monitoring or tracking?

P: Yes and no. We have statistics on what we did with the children in terms of interventions, and also how many children completed the program. This is a program that lasts four weeks, so the children can stay the whole time, but they can also leave early. If there are extreme conflicts or escalations, some children have to go back home if they endanger themselves or others. So the statistics cover how many people completed the program and how many left early. That is what we do in terms of progress tracking.

I: So it's general population tracking, not on an individual basis?

P: More like what we did with all the children: how many interventions of type A, how many of type B. We do put that in a chart for every individual child, but it's just to get it summarized, and then it gets added up and included in a yearly report. It's basically more about justifying to donors why this is necessary and effective. There's no long-term tracking.

I: The patients are done either early if something happened, or if they're better?

P: I wouldn't say cured. It's more like getting a break from whatever is burdening them at home, a restorative environment for four weeks. I'm not sure if you have something like that in the Netherlands. It's like a holiday but with a clear objective, psychological support, and pedagogical support. For example, if you have a health condition like a respiratory disease, you can also go to an institution for four weeks to improve your health. You're not being cured.

I: Then the next question is about administrative and practical support. How do you feel about AI generating drafts or reports, or assisting with scheduling and reminders?

P: Oh yes, please. This is such a big issue in our organization, communication and exchange of information. I think that would be the greatest aid this place could get. There are so many miscommunications and arguments resulting from faulty human information sharing or no information sharing. I think AI would be such a blessing.

I: You said this would be the greatest, but it cut out a bit. What did you say?

P: That would be the greatest aid for our organization in this setting. So much conflict happens between colleagues, and so much time and resources are lost because of human faults in communication and information distribution. I think that would be awesome.

I: Would you trust AI-generated reports, or would you need to review everything carefully?

P: Not 100%. I would trust AI if I could review it afterwards. I know it makes mistakes, but generally I think it does a good job.

I: How much time do you think this could save you?

P: So many hours. I can't really say exactly, but I work around 40 hours a week, so probably between 5 and 10 hours.

I: What concerns would you have about accuracy and liability?

P: AI makes mistakes, but I'd be more concerned about data protection, especially with a program like ChatGPT. My main concern is how the data is stored and used. Otherwise, as long as I can check it, I feel in control and responsible. I don't think it's the AI's responsibility.

I: So liability still lies with the psychologist using the AI?

P: Yes, one hundred percent.

I: Then for the next question, this is about psychoeducation and between-session tools. I'm not sure if this applies to you, but what are your thoughts on using AI for psychoeducation or therapeutic exercises between sessions?

P: I think that's a great idea. Psychoeducation can easily be delivered through AI, and I think it can do that efficiently and in an engaging way. Between-session practice is generally very useful for repetition and for stabilizing behaviors or alternatives to behaviors in treatment. You can probably do that at low cost at home, very easily accessible. I think that would be great.

I: Are you doing any between-session support or psychoeducation that you're already using with your clients?

P: Sometimes I give the children homework for our next sessions. This is Gen Alpha, a very tech-savvy generation. They want to be constantly on their phone, which I don't think is good, and we try to limit that by having phone times. But in real life, if they weren't here, it would be really good if they could access support through technology between sessions. I think that would work for that generation specifically, though for older people it's a bit trickier.

I: So during treatment they don't have that much access to their phone?

P: They have access but in a limited way, so some hours of the day they have the phone and some hours they don't. I could give them homework for when they're on their phone, but I assume that in those limited hours they wouldn't want to do psychological homework and would

rather chat or watch YouTube. So I think that wouldn't work in this context. But generally I think there are very good apps for this kind of thing, like for smoking cessation for example.

I: So what if the AI provides information that contradicts your treatment approach?

P: I think I would have the responsibility to look up the latest research and inform myself, and if the AI is wrong there should be a way to report that or send feedback to correct it. But it's still my responsibility to be aware of that and educate myself further. You cannot let the AI do your job completely.

I: Would you worry about clients relying too much on AI between sessions?

P: I think it depends on the client, whether that would be to your benefit all the time or not. Maybe it could be limited in some way, for example through an app feature. I don't want to say limit the client, but it depends on the profile of the patient. For example, if you have OCD or anxiety that could go the wrong way, or an addiction pattern.

I: And limiting might just be time limiting?

P: If you do an in-app session between your psychological sessions, and once you've completed that you cannot progress further, this is what you can do and no more. Something like that. But honestly I have so little knowledge on AI and technology in general, so maybe I'm not the best person to ask.

I: I've had a wide range of people who think they have knowledge on AI or don't. What's most important is figuring out what psychologists think, because if we find out that psychologists don't know much about AI, that's also a finding. So how would you monitor what the AI is telling your clients?

P: I think it has to be pre-approved, with a lot of rigorous testing beforehand. But that is difficult because AI will probably generate individually. I'm honestly not sure how you could intervene in that process.

I: Then the next question is about risks and limitations. What risks or concerns do you have about using AI in clinical work?

P: Data protection, not knowing where the data goes or who has control over it. I have a friend in law who told me there is actually very little regulation in that area, which is a concern. Also the accuracy of information, and still having control over what the AI delivers to a client. That is tricky, and I wouldn't know a way around that right now. Also loss of jobs.

I: With data privacy in mind, what information do you think would need to be most secure?

P: Especially diagnostic information, because in our society if that information leaks it can harm your career or basically any part of your life. It's similar to medical information generally. And just vulnerability, if a client trusts you with things like that.

I: So in general, any diagnostics or vulnerability connected with identifying traits, like name or where they live.

P: Yeah.

I: What concerns do you have about transparency and explainability of AI decisions?

P: What I've seen with ChatGPT is that it can explain the decisions or output it gives, which makes me feel more secure with it. But I think it would be good to have a second instance constantly checking the responses, a feedback loop of sorts. I would be a bit concerned about that. But generally when I check the content, most of the time it's correct, professionally and privately. Though not always, for example I booked a [TICKET] and was asking about the lineup, and it was always wrong.

I: Oh yeah, makes sense.

P: And that is quite easily accessible online if you look it up.

I: Have you thought about issues like bias in AI?

P: The AI being biased or me being biased?

I: Both.

P: Well, I'm a human, I'm biased. With the AI, I've tried to test it a couple of times. ChatGPT comes from the US, so I was asking questions about opinions about the government, and I found it not to be particularly biased, but a bit outdated in some outputs. Generally quite unbiased, perhaps a tiny bit.

I: How did you check for this bias?

P: I would ask questions about Donald Trump, whether he's a felon, whether he's convicted, whether he's sexist. That's what I asked the AI.

I: The next question is about client perspective and therapeutic alliance. How do you think clients respond to AI being used in their treatment?

P: Depends on the generation. I think younger people would be quite open to it.

I: So younger people would probably be open to it, and older people not as much?

P: Yeah, I think older people would be more distrusting because it's unfamiliar and new. And historically, with all the experience that a lot of older people have, understandably they can be more distrusting. Not everyone, but some people.

I: Can you give a score of how willing people would be to use AI in their treatment?

P: It depends on what population you look at, but I think generally, considering that we're moving forward, I would say a six or seven.

I: How might this affect the therapeutic alliance?

P: What do you mean by that specifically?

I: The therapeutic alliance is the agreement between you and your client to work toward getting better. If you're introducing AI into this relationship it might change the dynamic.

P: There is a possibility that this could introduce another instance, but it also depends on the person. Some AI, like ChatGPT, is made to sound like a human, and that could create a sense that it's not just a two-person alliance. I think that could complicate things, depending on how the AI is used.

I: The last question is about professional identity, autonomy and future conditions. Which aspects of your work should remain strictly human, and what would you need to feel confident about using AI safely in practice?

P: Everything regarding organizing and communications with colleagues, I think that can just be email communications, setting appointments and things like that. I think that's totally fine for AI. The session itself should be human interaction. Screening could be a combination of both, and documenting as well because you need to check it. I think it's always a combination rather than one or the other.

I: So why do you think these aspects need to stay human or at least partially human?

P: Because some things you cannot replace with technology. Just that feeling you have, the expressions you pick up on in human interactions, these very subtle things that technology cannot replicate.

I: Is there anything else you want to add?

P: Nothing.